



Why Justice Is Good for Our Health: The Social Determinants of Health Inequalities

Author(s): Norman Daniels, Bruce P. Kennedy and Ichiro Kawachi

Source: *Daedalus*, Fall, 1999, Vol. 128, No. 4, Bioethics and Beyond (Fall, 1999), pp. 215-251

Published by: The MIT Press on behalf of American Academy of Arts & Sciences

Stable URL: https://www.jstor.org/stable/20027594

JSTOR is a not-for-profit service that helps scholars, researchers, and students discover, use, and build upon a wide range of content in a trusted digital archive. We use information technology and tools to increase productivity and facilitate new forms of scholarship. For more information about JSTOR, please contact support@jstor.org.

Your use of the JSTOR archive indicates your acceptance of the Terms & Conditions of Use, available at https://about.jstor.org/terms



American Academy of Arts & Sciences and *The MIT Press* are collaborating with JSTOR to digitize, preserve and extend access to *Daedalus*

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

Norman Daniels, Bruce P. Kennedy, and Ichiro Kawachi

Why Justice is Good for Our Health: The Social Determinants of Health Inequalities

JUSTICE AND HEALTH INEQUALITIES

WE HAVE KNOWN for over 150 years that an individual's chances of life and death are patterned according to social class: the more affluent and educated people are, the longer and healthier their lives.[1] These patterns persist even when there is universal access to health care—a fact quite surprising to those who think financial access to medical services is the primary determinant of health status. In fact, recent cross-national evidence suggests that the greater the degree of socioeconomic inequality that exists within a society, the steeper the *gradient* of health inequality. As a result, middle-income groups in a less equal society will have worse health than comparable or even poorer groups in a society with greater equality. Of course, one cannot infer causation from correlation, but there are plausible hypotheses about pathways that link social inequalities to health. Even if more work remains to be done to clarify the exact mechanisms, it is not unreasonable to talk here about the social "determinants" of health.[2]

Norman Daniels is Goldthwaite Professor in the department of philosophy and professor of medical ethics in the department of community medicine at Tufts University.

Bruce P. Kennedy is deputy director of the division of public health practice at Harvard School of Public Health.

Ichiro Kawachi is director of the Harvard Center for Society and Health and associate professor of health and social behavior in the department of health and social behavior at Harvard School of Public Health.

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

When is an inequality in health status between different socioeconomic groups unjust?[3] An account of justice should help us determine which inequalities are unjust and which are tolerable. Many who are untroubled by some kinds of inequality are particularly troubled by health inequalities. They believe a socioeconomic inequality that otherwise seems just becomes unjust if it contributes to inequalities in health. But is every health inequality that results from unequally distributed social goods unjust? If there is an irreducible health gradient across socioeconomic groups, does that make the very existence of those inequalities unjust? Alternatively, are some health inequalities the result of acceptable trade-offs? Perhaps they are simply an unfortunate by-product of inequalities that work in other ways to help worse-off groups. For example, it is often claimed that permitting economic inequality provides incentives to work harder, thereby stimulating growth that will ultimately benefit the poorest groups. To whom must these trade-offs be acceptable if we are to consider them just? Are they acceptable only if they are part of a strategy aimed at moving the situation toward a more just arrangement? Does it matter in our judgments about justice exactly how social determinants produce inequalities in health status?

These are hard questions. Unfortunately, they have been almost completely ignored within the field of bioethics, as well as within ethics and political philosophy more generally. Bioethics has been quick to focus on exotic new medical technologies and how they might affect our lives. It has paid considerable attention to the doctor-patient relationship and how changes in the health-care system affect it. With some significant exceptions, it has not looked "upstream" from the point of delivery of medical services to the role of the health-care system in delivering improved population health. It has even more rarely looked further upstream to social arrangements that determine the health achievement of societies.[4]

This omission is quite striking, since a concern about "health equity" and its social determinants has emerged as an important consideration in the policies of several European countries over the last two decades.[5] The World Health Organization (WHO) has devoted increasing attention to inequalities in health

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

status and the policies that cause or mitigate them. So have research initiatives, such as the Global Health Equity Initiative, funded by the Swedish International Development Agency and the Rockefeller Foundation.

In what follows, we attempt to fill this bioethical gap by addressing some of these questions about justice and health inequalities. Rather than canvass answers that might be extracted from various competing theories of justice, however, we shall work primarily within the framework of John Rawls's theory of "justice as fairness" and the extension one of us has made of it to health care, probing the resources of that theory to address these issues.[6] Our contention is that Rawls's (extended) theory provides, albeit unintentionally, a defensible account of how to distribute the social determinants of health fairly. If we are right, this unexpected application to a novel problem demonstrates a fruitful generalizability of the theory analogous to the extension in scope or power of a nonmoral theory, and permits us to think more systematically across the disciplines of public health, medicine, and political philosophy.

The theory of justice as fairness was formulated to specify terms of social cooperation that free and equal citizens can accept as fair. Specifically, it assures people of equal basic liberties, including equal access to political participation; guarantees a robust form of equal opportunity; and imposes significant constraints on inequalities. Together, these principles aim at meeting the "needs of free and equal citizens," a form of egalitarianism Rawls calls "democratic equality."[7] A crucial component of democratic equality is providing all with the social bases of self respect and a conviction that prospects in life are fair. As the empirical literature demonstrates, institutions conforming to these principles together focus on several crucial pathways through which many researchers believe inequality works to produce health inequality. Of course, this theory does not answer all of our questions about justice and health inequality, since there are some crucial points on which it is silent, but it does provide considerable guidance on central issues.

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

SOCIAL DETERMINANTS OF HEALTH: SOME BASIC FINDINGS

There are four central findings in the literature on the social determinants of health, each of which has implications for an account of justice and health inequalities. First, the income/health gradients we observe are not the result of some fixed or determinate laws of economic development, but are influenced by policy choices. Second, the income/health gradients are not just the result of the deprivation of the poorest groups. Rather, a gradient in health operates across the whole socioeconomic spectrum within societies, such that the slope or steepness of the income/health gradient is affected by the degree of inequality in a society. Third, relative income or socioeconomic status (SES) is as important as, and may be more important than, the absolute level of income in determining health status, at least once societies have passed a certain threshold. Fourth, there are identifiable social and psychosocial pathways through which inequality produces its effects on health (and only modest support for "health selection," the claim that health status determines economic position).[8] These causal pathways are amenable to specific policy choices that should be guided by considerations of justice.

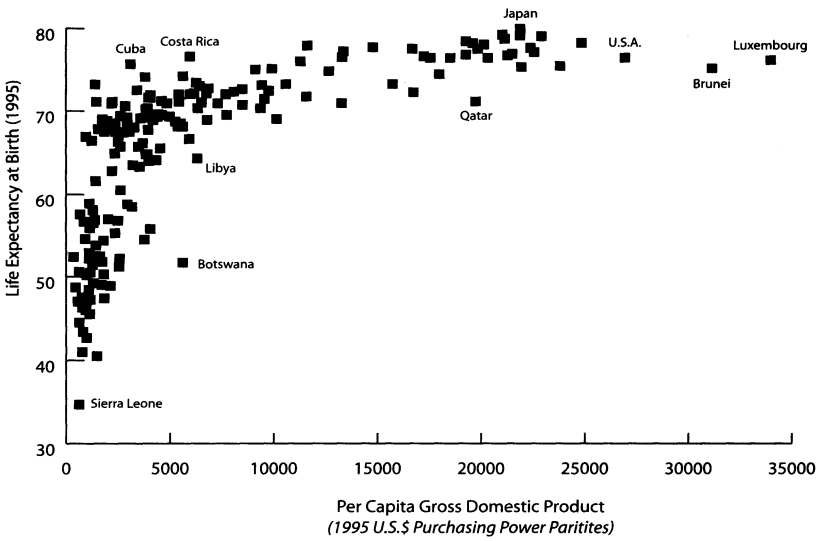
Cross-National Evidence on Health Inequalities

The pervasive finding that prosperity is related to health, whether measured at the level of nations or at the level of individuals, might lead one to the conclusion that these "income/health gradients" are inevitable. They might seem to reflect the natural ordering of societies along some fixed, idealized teleology of economic development. At the individual level, the gradient might appear to be the result of the natural selection of the most "fit" members within a society who are thus better able to garner socioeconomic advantage.

Despite the appeal and power of these ideas, they run counter to the evidence. Figure 1 shows the relationship between the wealth and health of nations as measured by per capita gross domestic product (GDPpc) and life expectancy. There is a clear association between GDPpc and life expectancy, but only up to

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

Figure 1: Relationship between country wealth and life expectancy



Source: United Nations Human Development Report Statistics, 1998.

a point. The relationship levels off beyond about $8,000–$10,000 GDPpc, with virtually no further gains in life expectancy.

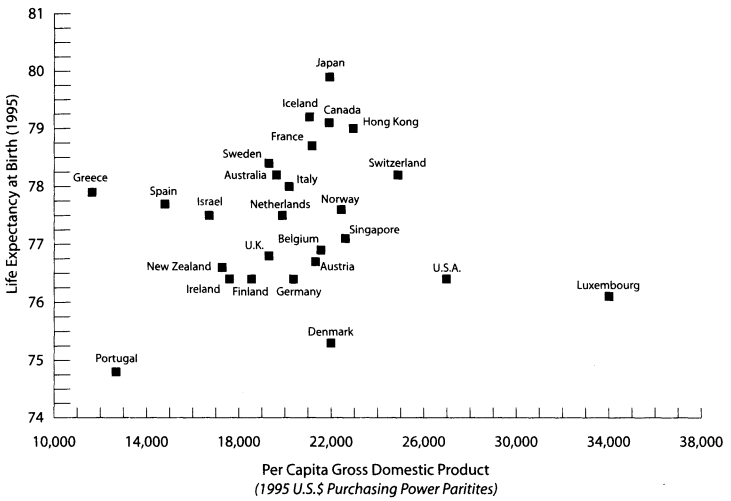
This leveling effect is most apparent among the advanced industrial economies (see Figure 2), which largely account for the upper tail of the curve in Figure 1. The leveling of the relationship between wealth and health is true *within* individual countries as well.

Closer inspection of these two figures points out some startling discrepancies. Though Cuba and Iraq are equally poor (GDPpc about $3,100), life expectancy in Cuba exceeds that in Iraq by 17.2 years. The difference between the GDPpc for Costa Rica and the United States, for example, is enormous (about $21,000), yet Costa Rica's life expectancy exceeds that of the United States (76.6 versus 76.4). In fact, despite being one of the richest nations on the globe, the United States performs rather poorly on health indicators.

Taken together, these observations support the notion that the relationship between economic development and health is not fixed, and that the health achievement of nations is medi-

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

Figure 2: Per capita gross domestic product, 1995 U.S.$ purchasing-power parities



Source: United Nations Human Development Report Statistics, 1998.

ated by processes other than wealth. To account for the cross-national variations in health, it is apparent that other factors—such as culture, social organization, and government policies—are significantly involved in the determination of population health, and that variations in these factors may go some distance toward explaining the differences in health outcomes between nations.

If we are right that the health of nations does not reflect some inevitable natural order but that it reflects policy choices—or features of society that are amenable to change via policies—then we must ask which of these policies are just.

Individual Socioeconomic Status and Health

At the individual level, numerous studies have documented what has come to be known as the "socioeconomic gradient." On this gradient, each increment up the socioeconomic hierarchy is associated with improved health outcomes over the rung

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

below.[9] It is important to observe that this relationship is not simply a contrast between the health of the rich and the poor but is observed across all levels of socioeconomic status. What is particularly notable about the SES gradient is that it does not appear to be explained by differences in access to health care. Steep gradients have been observed even among groups of individuals, such as British civil servants, with adequate access to health care, housing, and transport.[10]

Importantly, the steepness of the gradient varies substantially across societies. Some societies show a relatively shallow gradient in mortality across SES groups. Others, with comparable or even higher levels of economic development, show much steeper gradients in mortality rates across the socioeconomic hierarchy. The determining factor in the steepness of the gradient appears to be the extent of income inequality in a society. Thus, middle-income groups in a country with high income inequality may have lower health status than comparable or even poorer groups in a society with less income inequality. We find the same pattern within the United States when we examine state and metropolitan area variations in inequality and health outcomes.[11]

Relative Income and Health

The apparent connection between the distribution of income in a society and the level of health achievement of its members is a relatively recent finding.[12] Simply stated, it is not just the size of the economic pie but how the pie is shared that matters for population health. It is not the absolute deprivation associated with low economic development (lack of access to the basic material conditions necessary for health such as clean water, adequate nutrition and housing, and general sanitary living conditions) that explains health differences between developed nations, but the degree of *relative* deprivation within them. Relative deprivation refers not to a lack of the "goods" that are basic to survival, but rather to a lack of sources of self-respect that are deemed essential for full participation in society.

Numerous studies have provided support for the relative-income hypothesis, both between and within nations.[13] This finding helps to explain the anomalies highlighted in Figures 1

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

and 2. Much of the variation in life expectancy for the wealthy countries in the upper tail of Figure 1 is explained by income distribution: countries with more equal income distributions, such as Sweden and Japan, have higher life expectancies than do countries such as the United States, regardless of GDPpc. Furthermore, countries with much lower GDPpc, such as Costa Rica, appear to be able to obtain their remarkably high life expectancy through a more equitable distribution of income.

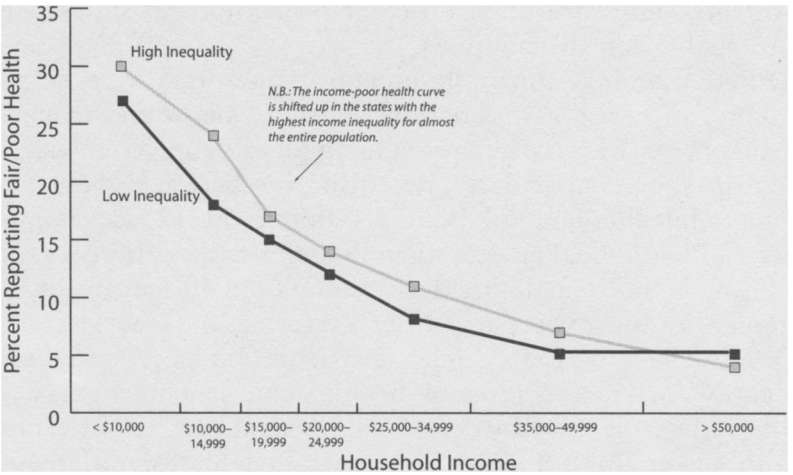
Within the United States, income inequality accounts for about 25 percent of the between-state variance in age-adjusted mortality rates independent of state median income.[14] Moreover, the size of this relationship is not trivial. A recent study across U.S. metropolitan areas, rather than states, found that areas with high income inequality had an excess of death compared to areas of low inequality that was equivalent in magnitude to all deaths due to heart disease.[15]

While most of the evidence so far has been accumulated from cross-sectional data, time-trend data support similar conclusions. Widening income differentials in the United States and the United Kingdom appear to be related to a slowing down of life-expectancy improvements. In many of the poorest areas of the United Kingdom, mortality for younger age cohorts has actually increased during the same period that income inequality widened.[16] In the United States, states with the highest income inequality showed slower rates of life-expectancy improvement compared to states with more equitable income distributions between 1980 and 1990.[17]

As we noted above, income distribution appears to affect the health of populations by shifting the slope of the curve relating individual income to health. This can be clearly seen in Figure 3, where the prevalence of self-reported fair/poor health is higher (and the gradient steeper) in almost every income group for those living in states with the highest income inequality.[18] Nearly identical patterns have been found for individual mortality rates.[19]

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

Figure 3: Self-related health and individual household income in the U.S.



Source: Ichiro Kawachi and Bruce P. Kennedy, "Income Inequality and Health: Pathways and Mechanisms," *Health Services Research* 34 (1999): 215–227.

Pathways Linking Social Inequalities to Health Inequalities

Our final contention is that there are plausible and identifiable pathways through which social inequalities produce inequalities in health. Some of these occur at the societal level, where income inequality creates a pattern for the distribution of social goods, such as public education, thereby affecting access to life opportunities—which are, in turn, strong determinants of health.

The evidence for these associations, while fairly new, is quite striking. In the United States, the most inegalitarian states with respect to income distribution invest less in public education, have larger uninsured populations, and spend less on social safety nets.[20] Differences in human capital investment, typically measured in terms of educational spending and (more important) educational outcomes, are particularly striking. Even when controlling for median income, income inequality explains about 40 percent of the between-state variation in the percentage of children in the fourth grade who fall below the basic reading level. Similarly strong associations are seen in the percentage of

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

high-school dropout rates. It is quite evident from these data that educational opportunities for children in high income-inequality states are quite different from those in states with more egalitarian distributions.

Differential investment in human capital is also a strong predictor of health across nations. Indeed, one of the strongest predictors of life expectancy among developing countries is adult literacy, particularly the disparity between male and female adult literacy. For example, among the 125 developing countries with GDPpc less than $10,000, the difference between male and female literacy accounts for 40 percent of the variance in life expectancy after factoring out the effect of GDPpc. The fact that gender disparities in access to basic education drives the level of health achievement emphasizes further the role of broader social inequalities in patterning health inequalities. Indeed, in the United States, differences between the states in women's status—measured in terms of their economic autonomy and political participation—are strongly correlated with female mortality rates.[21]

These societal mechanisms are tightly linked to the political processes that influence government policy. One way that income inequality affects health and social welfare appears to be through its role in undermining civil society. Income inequality erodes social cohesion, as measured by levels of social mistrust and participation in civic organizations, both of which are features of civil society.[22] Lack of social cohesion is in turn reflected in significantly lower participation in political activity (e.g., voting, serving in local government, volunteering for political campaigns, etc.), thus undermining the responsiveness of government institutions in addressing the needs of the worst off in society. This is demonstrated by the human capital investment data presented earlier, but it is also reflected by the lack of investment in human security. States with the highest income inequality, and thus the lowest levels of social capital and political participation, are far less generous in the provision of social safety nets. For example, the correlation between social capital, as measured by low interpersonal trust, and the maximum welfare grant as a percent of state per capita income is –0.76.[23]

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

How can the points we have highlighted from scientific literature on social determinants be integrated into our views about the moral acceptability of health inequalities? Historically, disciplinary boundaries have stood as an obstacle to an integrated perspective. The social-science and public-health literature sharpens our understanding of the causes of health inequalities, but it contains no systematic way to evaluate the overall fairness of those inequalities and the socioeconomic inequalities that produce them. The philosophical literature has produced theories aimed at evaluating socioeconomic inequalities, but it has tended to ignore health inequalities and their causes. To produce an integrated view, we shall need the resources of a more general theory of justice. We can better see the need for such a theory if we first examine an analysis of "health inequities" that has been developed within a policy-oriented public-health literature.

HEALTH INEQUALITIES AND INEQUITIES

When is a health inequality between two groups "inequitable"? This version of our earlier question about health inequalities and justice has been the focus of European and WHO efforts, as noted above. One initial answer that has been influential in the WHO programs is the claim, posited in writings by such researchers as Margaret Whitehead and Goran Dahlgren, that health inequalities count as inequities when they are *avoidable*, *unnecessary*, and *unfair*.[24] If we can agree on what is avoidable, unnecessary, and unfair—and if this analysis is correct—then we can agree on which inequalities are inequitable.

Age, gender, race, and ethnic differences in health status exist that are independent of socioeconomic differences, and they raise distinct questions about equity or justice. For example, should we view the lower life expectancy of men compared to women in developed countries as an inequity? If it is rooted in biological differences that we do not know how to overcome, then, according to this analysis, it is not avoidable and therefore not an inequity. This is not an idle controversy: taking average, rather than gender-differentiated, life expectancy in developed countries as a benchmark will yield different

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

estimates of the degree of inequity women face in some developing countries. In any case, the analysis of inequity is here only as good as our understanding of what is avoidable or unnecessary.

The same point applies to judgments about fairness. Is the poorer health status of some social classes or ethnic groups that engage in heavy tobacco and alcohol use unfair? We may be inclined to say it is not unfair, provided that participation in the risky behaviors or their avoidance is truly voluntary. But if many people in a cultural group or class behave similarly, there may also be factors at work that reduce how voluntary their behavior is and how much responsibility we should ascribe to them for it.[25] The analysis thus leaves us with the unresolved complexity of these judgments about responsibility and, as a result, with disagreements about fairness (or avoidability).

The poor in many countries lack access to clean water, sanitation, adequate shelter, basic education, vaccinations, and prenatal and maternal care. As a result of some or all of these factors, there are infant mortality differences between these and richer groups. Since social policies could supply the missing determinants of infant health, the inequalities must be regarded as avoidable. Are these inequalities also unfair? Most of us would immediately think they are, perhaps from a view that policies that not only countenance but sustain poverty are unjust. Social policies that compound poverty with a lack of access to the determinants of health may be viewed as doubly unfair. Of course, libertarians would disagree. They would insist that what is merely unfortunate is not unfair; in their view, society has no obligation of justice (as opposed to charity) to provide the poor with what they are missing.

Many of us might be inclined to reject the libertarian view as in itself unjust because of the dramatic conflict it opens with our beliefs about poverty and our social obligations to meet people's basic needs. The problem becomes more complicated, however, when we remember one of the basic findings from the literature on social determinants: we cannot eliminate health inequalities simply by eliminating poverty. Health inequalities persist even in societies that provide the poor with access to all of the determinants of health noted above, and they persist as a gra-

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

dient of health throughout the social hierarchy, not just between the very poorest groups and those above them.

Faced with this realization, many of us may have to reexamine what we believe about the justice of other sorts of socioeconomic inequalities. Unless we believe that all socioeconomic inequalities (or at least all inequalities we did not choose) are unjust—and very few embrace such a radically egalitarian view—then we must consider more carefully the problem posed by the health gradient and the fact that it is made steeper under more unequal social arrangements. Judgments about fairness—to which many, rightly or wrongly, feel confident in appealing when rejecting the libertarian position—give less guidance than perhaps had been expected in thinking about the broader issue of the social determinants of health inequalities. Indeed, we may even believe that some degree of socioeconomic inequality is unavoidable or even necessary, and therefore not unjust.

JUSTICE AS FAIRNESS AND HEALTH INEQUALITIES

One reason we develop general ethical theories, including theories of justice, is to provide a framework within which to resolve important disputes of the sort we have just raised about conflicting moral beliefs or intuitions. For example, in *A Theory of Justice* John Rawls sought to leverage the relatively broad liberal agreement on principles guaranteeing certain equal basic liberties into an agreement on a principle limiting socioeconomic inequalities—a matter on which liberals disagree considerably.[26] His strategy was to show that a social contract designed to be fair to free and equal people ("justice as [procedural] fairness") would not only justify the choice of those equal basic liberties but would also justify the choice of principles guaranteeing equal opportunity and limiting inequalities to those that work to make the worst-off groups fare as well as possible.

Our contention is that Rawls's account, though developed to answer this general question about social justice without specifically contending with issues of disease or health, turns out to provide useful principles for the just distribution of the social determinants of health. To simplify the construction of his

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

theory, Rawls assumed his contractors would be fully functional over a normal life span—i.e., that no one would become ill or die prematurely.

This idealization itself provides a clue about how to extend this theory to the real world of illness and premature death. The goal of public health and medicine is to keep people as close as possible to the ideal of normal functioning, under reasonable resource constraints. (Resources are necessarily limited since maintaining health cannot be our only social good or goal.) Since the maintenance of normal functioning makes a limited but significant contribution to protecting the range of opportunities open to all individuals, it is plausible to see a principle guaranteeing equality of opportunity satisfying the condition of fairness as the appropriate principle to govern the distribution of health care, broadly construed to include primary and secondary preventive health as well as medical services.[27]

What is particularly appealing about examining the social determinants of health inequalities from the perspective of Rawls's theory is that the theory is egalitarian in orientation and yet justifies certain inequalities that might contribute to health inequalities. In addition, Norman Daniels's extension of Rawls links the protection of health to the protection of equality of opportunity, again setting up the potential for internal conflict. To see whether this combination of features leads to insight into the problem or simply to contradictions in the theory, we must examine the issue in more detail.

How does Rawls justify socioeconomic inequalities? Why would free and equal contractors not simply insist on strictly egalitarian distribution of all social goods, just as they insist on equal basic liberties and equal opportunity?

Rawls's answer is that it is irrational for contractors to insist on equality if doing so would make them worse off. Specifically, he argues that contractors would choose his Difference Principle, which permits inequalities provided that they work to make the worst-off groups in society as well off as possible.[28] The argument for the Difference Principle appears to suggest that relative inequality is less important than absolute well-being, a suggestion that is in tension with other aspects of Rawls's view. Thus he also insists that inequalities allowed by

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

the Difference Principle should not undermine the value of political liberty and the requirements of fair equality of opportunity. The priority given these other principles over the Difference Principle thus limits the inference that Rawls has no concern about relative inequality. Specifically, as we shall see, these principles work together to constrain inequality and to preserve the social bases of self-respect for all.

Two points will help avoid misunderstanding of the Difference Principle and its justification. First, it is not a mere "trickle-down" principle but one that requires maximal flow in the direction of helping the worst-off groups. The worst off, and then the next worst off, and so on (Rawls calls this "chain connectedness"[29]) must be made as well off as possible, not merely just somewhat better off, as a trickle-down principle implies. The Difference Principle is thus much more demanding than a principle that would permit any degree of inequality provided there was some "trickle" of benefits to the worst off. Indeed, it is more egalitarian than alternative principles that merely assure the worse off a "decent" or "adequate" minimum. From what we have learned about the social determinants of health, the more demanding Difference Principle would also produce less health inequality than any proposed alternative principles that allow inequalities. By flattening the health gradient, it would also benefit middle-income groups, not simply the poorest. In this regard, its benefits are important beyond the level at which we have helped the worst off to achieve "sufficiency." This point provides a reply to those who suggest that the Difference Principle has no appeal once the worst off are sufficiently provided for.[30]

Second, when contractors evaluate how well off the principles they choose will make them, they judge their well-being by an index of "primary social goods."[31] The primary social goods, which Rawls thinks of as the "needs of citizens," include liberty, powers, opportunities, income and wealth, and the social bases of self-respect. (These objective measures of well-being should be contrasted with measures of happiness or desire-satisfaction that are familiar from utilitarian and welfare economic perspectives.) In his exposition of the Difference Principle, Rawls illustrates how it will work by asking us to con-

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

sider only the simpler case of income inequalities. In doing so, he assumes that the level of income will correlate with the level of other social goods on the index.

This simplification should not mislead us, for, in crucial cases, the correlation may not obtain. For example, let us suppose that having "democratic" control over one's workplace is crucial to self-realization and the promotion of self-esteem.[32] Suppose further that hierarchical workplaces are more efficient than democratic ones, so that a system with hierarchical workplaces would have resources to redistribute that meant higher incomes for worst-off workers than democratic workplaces would permit. Then the Difference Principle does not clearly tell us whether the hierarchical workplace contains allowable inequalities since the worst off are better off in some ways but worse off in others. Without knowing the weighting of items in the index, we cannot use it to say clearly what inequalities are permitted. When we are evaluating which income inequalities are allowable, by asking which ones work to make the worst-off groups as well off as possible, we must, in any case, judge how well off groups are by reference to the whole index of primary goods—not simply the resulting income.

This point is of particular importance in the current discussion. Daniels's extension of Rawls treats health status as a determinant of the range of opportunity open to individuals. Since opportunity is included in the index, the effects of health inequalities are thereby included in the index.

Rawls says very little, however, about how items in the index are to be weighted. Therefore we have little guidance about how these primary goods are to be traded off against each other in its construction. This silence pertains not only to the use of the index in the contract situation, but also to its use, as we will examine more closely below, by a legislature trying to apply the principles of justice in a context in which many specific features of a society are known.

We can now say more directly why justice, as described by Rawls's principles, is good for our health. To understand this claim, let us start with the ideal case, a society governed by Rawls's principles of justice that seeks to achieve "democratic equality."[33] Consider what it requires with regard to the distri-

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

bution of the social determinants of health. In such a society, all are guaranteed equal basic liberties, including the liberty of political participation. In addition, there are institutional safeguards aimed at assuring all, richer and poorer alike, the worth or value of political participation rights. Without such assurance, basic capabilities of citizens cannot develop. The recognition that all citizens must have these capabilities protected is critical to preserving self-esteem, in Rawls's view. Since there is evidence, as we have seen, that political participation is itself a social determinant of health, the Rawlsian ideal assures institutional protections that counter the usual effects of socioeconomic inequalities on participation and thus on health.

The Rawlsian ideal of democratic equality also involves conformity with a principle guaranteeing a fair distribution equality of opportunity. Not only are discriminatory barriers prohibited by the principle; it requires as well robust measures aimed at mitigating the effects of socioeconomic inequalities and other social contingencies on opportunity. In addition to equitable public education, such measures would include the provision of developmentally appropriate day care and early childhood interventions intended to promote the development of capabilities independently of the advantages of family background. Such measures match or go beyond the best models we see in European efforts at day care and early childhood education. We also note that the strategic importance of education for protecting equal opportunity has implications for all levels of education, including access to graduate and professional education.

The equal opportunity principle also requires extensive public-health, medical, and social-support services aimed at promoting normal functioning for all.[34] It even provides a rationale for the social costs of reasonable accommodation to incurable disabilities, as required by the Americans with Disabilities Act.[35] Because the principle aims at promoting normal functioning for *all* as a way of protecting opportunity for all, it simultaneously aims at both improving population health and reducing health inequalities. Obviously, this focus requires the provision of universal access to comprehensive health care, including public health, primary health care, and medical and social support services.

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

To act justly in health policy, we must have knowledge about the causal pathways through which socioeconomic (and other) inequalities work to produce differential health outcomes. Suppose we learn, for example, that structural and organizational features of the workplace inducing stress and a sense of a loss of control tend to promote health inequalities. We should then view the modification of those features of workplace organization as a public-health requirement of the equal-opportunity principle in order to mitigate their negative effects on health. It would thus be placed on a par with the requirement that we reduce exposures to toxins in the workplace.[36]

Finally, in the ideal Rawlsian society, the Difference Principle places significant restrictions on allowable inequalities in income and wealth.[37] The inequalities allowed by this principle (in conjunction with the principles assuring equal opportunity and the value of political participation) are probably more constrained than those we observe in even the most industrialized societies. If so, then the inequalities that conform to the Difference Principle would produce a flatter gradient of health inequality than we currently observe in even the more extensive welfare systems of northern Europe.

In short, Rawls's principles of justice regulate the distribution of the key social determinants of health, including the social bases of self-respect. There is nothing about the theory, or Daniels's extension of it, that should make us focus narrowly on medical services. Properly understood, justice as fairness tells us what justice requires in the distribution of all socially controllable determinants of health.

We still face a theoretical issue of some interest. Even if the Rawlsian distribution of the determinants of health flattens health gradients further than what we observe in the most egalitarian, developed countries, we must still expect a residue of health inequalities. In part, this may happen because we may not have adequate knowledge of all the relevant causal pathways or interventions that are effective in modifying them. The theoretical issue is whether the theory requires us to reduce *further* those otherwise justifiable inequalities because of the inequalities in health status they create.

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

We should not further reduce those socioeconomic inequalities if doing so reduces productivity to the extent that we can no longer support the institutional measures we already employ to promote health and reduce health inequalities. Our commitment to reducing health inequality should not require steps that threaten to make health worse for those with less-than-equal health status. So the theoretical issue reduces to this: would it ever be reasonable and rational for contractors to accept a tradeoff in which some health inequality is allowed in order to produce some nonhealth benefits for those with the worst health prospects?

We know that in real life people routinely trade health risks for other benefits. They do so when they commute longer distances for a better job or take a ski vacation. Some such trades raise questions of fairness. For example, when is hazard pay a benefit workers gain only because their opportunities are unfairly restricted, and when is it an appropriate exercise of their autonomy?[38] Many such trades we would not restrict, thinking it unjustifiably paternalistic to do so; others we see as unfair.

Rawlsian contractors, however, cannot make such trades on the basis of any specific knowledge of their own values. They cannot decide that their enjoyment of skiing makes it worth the risks to their knees or necks. To make the contract fair to all participants, and to achieve impartiality, Rawls imposes a thick "veil of ignorance" that blinds them to all knowledge about themselves, including their specific views of the good life. Instead, they must judge their well-being by reference to an index of primary social goods (noted earlier) that includes a weighted measure of rights, opportunities, powers, income and wealth, and the social bases of self-respect. When Kenneth Arrow first reviewed Rawls's theory, he argued that this index was inadequate because it failed to tell us how to compare the ill rich with the well poor.[39] Amartya Sen has argued that the index is insensitive to the way in which disease, disability, or other individual variations would create inequalities in the capabilities of people who had the same primary social goods.[40] By extending Rawls's theory to include health care through the equal opportunity account, some of Arrow's (and Sen's) criti-

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

cism is undercut.[41] But even raising our question about residual health inequalities reminds us that the theory says too little about the construction of the index to provide us with a clear answer.

One of Rawls's central arguments for singling out a principle protecting equal basic liberties and giving it (lexical) priority over his other principles of justice is his claim that once people achieve some threshold level of material well-being, they would not trade away the fundamental importance of liberty for other goods.[42] Making such a trade might deny them the liberty to pursue their most cherished ideals, including their religious beliefs, whatever they turn out to be. Can we make the same argument about trading health for other goods?

There is some plausibility to the claim that rational people would refrain from trades of health for other goods. Loss of health may preclude us from pursuing what we most value in life. We do, after all, see people willing to trade almost anything to regain health once they lose it. If we take this argument seriously, we might conclude that Rawls should give opportunity—considered as including the effects of health status—a heavier weighting in the construction of the index than income alone.[43] Such a weighting would mean that absolute increases in income that might otherwise have justified increasing relative income inequality, according to the Difference Principle, now fail to justify that inequality because of the negative effects on opportunity. Although the income of the worst off would increase, they would not be better off according to the whole (weighted) index of primary social goods, and so the greater inequality would not be permitted. Rawls's simplifying assumption about income correlating with other goods fails in this case (as it did in the hypothetical example about workplace democracy cited earlier).

Nevertheless, there is also strong reason to think that the priority given to health, and thus opportunity, is not as clear-cut as the previous argument implies—especially where the trade is between a *risk* to health and other goods that people highly value. Refusing to allow any (*ex ante*) trades of health risks for other goods, even when the background conditions on choice are otherwise fair, may seem unjustifiably paternalistic,

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

perhaps in a way that refusing to allow trades of basic liberties is not.

We propose a pragmatic route around this problem, one that has a precedent elsewhere in Rawls. Fairness in equality opportunity, Rawls admits, is only approximated even in an ideally just system, because we can only mitigate, not eliminate, the effects of family and other social contingencies.[44] For example, only if we were willing to violate widely respected parental liberties could we intrude into family life and "rescue" children from parental values that arguably interfere with equal opportunity. Similarly, though we give a general priority to equal opportunity over the Difference Principle, we cannot achieve complete equality in health any more than we can achieve completely equal opportunity. Even ideal theory does not produce perfect justice. Justice is always rough around the edges. Specifically, if we had good reason to think that "democratic equality" had flattened inequalities in accord with the principles of justice, then we might be inclined to think we had done as much as was reasonable to make health inequalities fair to all. The residual inequalities that emerge in conformance to the principles are not a "compromise" with what justice ideally requires; they are acceptable as just.

So far, we have been considering whether the theoretical question of a health tradeoff can be resolved from the perspective of individual contractors. Instead, suppose that the decision about such a tradeoff is to be made through the legislature in a society that conforms to Rawls's principles. Because those principles require effective political participation across all socioeconomic groups, we can suppose that groups most directly affected by any tradeoff decision have a voice in the decision. Since there is a residual health *gradient*, groups affected by the tradeoff include not only the worst off, but those in the middle as well. A democratic process that involved deliberation about the tradeoff and its effects might be the best we could do to provide a resolution of the unanswered theoretical question.[45]

In contrast, where the fair value of political participation is not adequately assured—and we doubt it is so assured in even our most democratic societies—we have much less confidence

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

in the fairness of a democratic decision about how to trade health against other goods. It is much more likely under actual conditions that those who benefit most from the inequalities, i.e., those who are better off, also wield disproportionate political power and will influence decisions about tradeoffs to serve their interests. It may still be that the use of a democratic process in nonideal conditions is the fairest resolution we can practically achieve, but it still falls well short of what an ideally just democratic process involves.

We have focused on Rawlsian theory because it provides, however unintentionally, a developed account of how to distribute the social determinants of health. Other, competing theories of justice, including some recent proposals about "equal opportunity for welfare or advantage," offer no similarly developed framework for distributing the key social determinants of health.[46] On the other hand, Sen's account of the importance of an egalitarian distribution of "capabilities" actually resembles the account offered by Daniels (and others) of equal opportunity and normal functioning more than it seems at first.[47] Elizabeth Anderson has imaginatively focused the discussion of capabilities on those needed if citizens are to have "democratic equality."[48] The result is a striking convergence with Rawls's view of democratic equality—although Rawls's ability to talk about the fair distribution of social determinants of health follows directly from his principles, whereas Anderson must appeal intuitively to an account of the capabilities needed by citizens.

POLICY IMPLICATIONS FOR A JUST DISTRIBUTION OF THE SOCIAL DETERMINANTS OF HEALTH

We earlier suggested that the analyses of Whitehead and Dahlgren of health inequities (inequalities that are avoidable and unfair) are useful, provided that we can agree on what counts as avoidable and unfair. We then suggested that the Rawlsian account of justice as fairness provides a fuller account of what is fair and unfair in the distribution of the social determinants of health. The theory provides a more systematic way to think about which health inequalities are inequities.

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

Compared to that ideal, most health inequalities that we now observe worldwide among socioeconomic and racial or ethnic groups are "inequities" that should be remedied. Even some countries with the shallowest health gradients, such as Sweden and England, have viewed their own health inequalities as unacceptable and have initiated policy measures to mitigate them. The broader efforts of the World Health Organization in this direction are, probably without exception, also aimed at true inequities.

A central policy implication of our discussion is that reform efforts to improve health inequalities must be intersectoral and not focused just on the traditional health sector. Health is produced not just by having access to medical prevention and treatment, but, to a measurably greater extent, by the cumulative experience of social conditions across the life course. In other words, by the time a sixty-year-old patient is brought to the emergency room to receive medical treatment for a heart attack, that encounter represents the result of bodily insults accumulated over a lifetime. Medical care is, figuratively speaking, "the ambulance waiting at the bottom of the cliff." Much of the contemporary discussion about increasing access to medical care as a means of reducing health inequalities misses this point. An emphasis on intersectoral reform will recognize the primacy of social conditions, such as access to basic education, levels of material deprivation, a healthy workplace environment, and equality of political participation in determining the health achievement of societies.[49]

Before saying more about intersectoral reform, we want to head off what from our view is a mistaken inference—namely that we should ignore medical services and health-sector reform because other steps have a bigger payoff. Even if we had a highly just distribution of the social determinants of health and of public-health measures, people would still become ill and need medical services. The fair design of a health system should give weight to meeting actual medical needs.

We might think of those with known needs and those who are ill as "identified victims," whereas we might think of those whose lives would be spared illness by robust public-health measures and a fair distribution of social determinants as "statis-

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

tical victims." Several theoretical perspectives, both utilitarian and nonutilitarian, would hold that we should consider all these lives impartially, judging statistical lives saved to be just as valuable or important as identified victims. Utilitarian approaches would push us to maximize net benefit by allocating resources from saving identified lives to saving statistical ones.

Other reasonable considerations, however, temper our inclination to reallocate in such an impartial way from identified to statistical victims. Many of us give some extra moral weight to the urgent needs of those already ill. Others, through their roles as medical providers, may legitimately believe that the good that they can achieve through their control over the delivery of medical care has a greater claim on them than the good that would be brought about by more indirect measures beyond their control. More generally, many of us will be connected as family members and friends to the identified victims and will feel that we have "agent-relative" obligations to assist them that supersede the obligations we have to more distant, statistical victims.

It is impossible to dismiss the relevance of these other considerations. Consequently, we do not draw the inference that impartiality or rationality considerations might seem to support: namely, that we should immediately reallocate resources away from medical services to public-health measures or a fairer distribution of social determinants in accordance with some algorithm based on the relative benefit, neutrally calculated, between statistical and identified lives. This is not to imply, however, that no reallocations are justifiable. No doubt some are, in light of what we have argued.[50]

What sorts of social policies should governments pursue in order to reduce health inequalities? Certainly, the menu of options should include equalizing access to medical care; but it should also include a broader set of policies aimed at equalizing individual life opportunities, such as investment in basic education, affordable housing, income security, and other forms of antipoverty policy. Though the connection between these broad social policies and health may seem somewhat remote, and though such policies are rarely linked to issues of health in our public-policy discussions, growing evidence suggests that they

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

should be so linked. The kinds of policies suggested by a social-determinants perspective encompass a much broader range of instruments than would be ordinarily considered for improving the health of the population. We discuss three such examples of social policies that hold promise of abating socioeconomic disparities in health: investment in early childhood development, improvement in the quality of the work environment, and reduction in income inequality.

The Case for Early Life Intervention

Growing evidence points to the importance of the early childhood environment in influencing the behavior, learning, and health of individuals later in the life course. Providing equal opportunities within a Rawlsian framework translates into intervening as early in life as possible. Several studies have demonstrated the benefits of early supportive environments for children. In the Perry High/Scope Project, children in poor economic circumstances were provided a high-quality early childhood development program between the ages of three and five.[51] Compared to the control group, those in the intervention group completed more schooling by age twenty-seven, were more likely to be employed, own a home, and be married with children, experienced fewer criminal problems and teenage pregnancies, and were far less likely to have mental-health problems.

Compensatory education and nutrition in the early years of life (as exemplified by the Head Start and WIC programs) have been similarly shown to yield important gains for the most disadvantaged groups. As part of Lyndon Johnson's War on Poverty, the U.S. government introduced two small compensatory-education programs—Head Start for preschoolers, and Chapter 1 (now Title 1) for elementary school children. Evaluations of these programs indicate that children who enroll in them learn more than those who do not. In turn, educational achievement is a powerful predictor of health in later life, partly because education provides access to employment and income, and partly because education has a direct influence on health behavior in adulthood, including diet, smoking, and physical activity.[52]

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

A similarly persuasive case can be made for nutritional supplementation in low-income women and children. An analysis of the National Maternal and Infant Health Survey found that participation of low-income pregnant women in the WIC program (the Special Supplemental Nutrition Program for Women, Infants, and Children) was associated with about a 40 percent reduction in the risk of subsequent infant death.[53] A mother's nutritional state affects her infant's chance of death not just in the first year of life but also throughout the life course. Thus, a woman's pre-pregnant weight is one of the strongest predictors of her child's birth weight, and, in turn, low birth weight has been linked with increased risks of coronary heart disease, hypertension, and diabetes in later life. It follows that investing in policies to reduce early adverse influences may produce benefits not only in the present, but also in the long run and for future generations.

The Case for Improving the Quality of the Work Environment

We alluded earlier to the finding that the health status of workers is closely linked to the quality of their work environment, specifically to the amount of control and autonomy available to workers on their jobs. Low-control work environments—such as monotonous, machine-paced work (e.g., factory assembly lines) or jobs involving little opportunity for learning and utilization of new skills (e.g., supermarket cashiers)—tend to be concentrated among low-income occupations. The work of Marmot and his colleagues has shown that social disparities in health arise partly as a consequence of the way labor markets sort individuals into positions of unequal authority and control.[54] Exposure to low-control, high-demand job conditions not only is more common in lower-status occupations, but also places workers at increased risk of hypertension, cardiovascular disease, mental illness, musculoskeletal disease, sickness absence, and physical disability.[55]

A growing number of international case studies has concluded that it is possible to improve the level of control in workplaces by several means: increasing the variety of different tasks in the production process; encouraging workforce participation in the production process; and allowing more

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

flexible work arrangements, such as altering the patterns of shift work, to make them less disruptive of workers' lives.[56] In some cases it may even be possible to redesign the workplace and enhance worker autonomy without affecting productivity, since sickness absence may diminish as a consequence of a healthier workplace.

The Case for Income Redistribution

Many of the measures suggested by the social-determinants perspective tend to fall into the category of antipoverty policy. However, research on the social determinants of health warns us that antipoverty policies *do not go far enough* in reducing unjust health disparities. Though none would disagree about putting priority on reducing the plight of the worst off, the fact is that health inequalities occur as a gradient: the poor have worse health than the near-poor, the near-poor fare worse than the lower middle class, the lower middle class do worse than the upper middle class, and so on up the economic ladder. Addressing the social gradient in health requires action above and beyond the elimination of poverty.

To address comprehensively the problem of health inequalities, governments must begin to address the issue of economic inequalities *per se*. As we noted above, growing international and intranational evidence suggests that the extent of socioeconomic disparities—that is, the size of the gap in incomes and assets between the top and bottom of society—is itself an important determinant of the health achievement of society, independent of the average standard of living.[57] Most importantly, economic disparities seem to influence the degree of equality in political participation (in the form of voting, donating to campaigns, contacting elected officials, and other forms of political activity): the more unequal the distribution of incomes and assets, the more skewed the patterns of political participation, and, consequently, the greater the degree of political exclusion of disadvantaged groups.[58]

Inequalities in political participation in turn determine the kinds of policies passed by national and local governments. For example, Kim Hill and his colleagues carried out a pooled time-series analysis for the fifty U.S. states from 1978 to 1990 to

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

examine the relationship between the degree of mobilization of lower-class voters at election time and the generosity of welfare benefits provided by state governments.[59] Even after adjusting for other factors that might predict state welfare policies—such as the degree of public liberalism in the state, the federal government's welfare cost-matching rate for individual states, the state unemployment rate and median income, and the state tax effort—robust relationships were found between the extent of political participation by lower-class voters and the degree of generosity of state welfare payments. In other words, *who participates* matters for political outcomes, and the resulting policies have an important impact on the opportunities for the poor to lead a healthy life.

For both of the foregoing reasons—that it yields a higher level of health achievement as well as greater political participation—the reduction of income disparity ought to be a priority of governments concerned about addressing social inequalities in health. Although the scope of this essay precludes further consideration here, a number of levers do exist by which governments could address the problem of income inequality, spanning from the radical (a commitment to sustained full employment, collective wage bargaining, and progressive taxation) to the incremental (expansion of the earned income tax credit, increased child care credit, and a raise in the minimum wage).[60]

Implications for International Development Theory

Our discussion has implications for international development theory, as well as for the economic choices confronted by industrialized countries. To the extent that income distribution matters for the health status of given populations, it is not obvious that giving strict priority to economic growth is the optimal strategy for maximizing social welfare. Raising everyone's income will improve the health status of the poor (the trickle-down approach), but not as much as paying attention to the *distribution* of the social product. Within the developing world, a comparison of the province of Kerala in India with more unequal countries like Brazil and South Africa illustrates this point. Despite having only a third to a quarter of the income of Brazil or South Africa (and thereby having a higher

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

prevalence of poverty in the absolute sense), the citizens of Kerala nonetheless live longer, most likely as a result of the higher priority that the government of Kerala accords to a fair distribution of economic gains.[61]

The real issue for developing countries is what *kind* of economic growth is salutary. Hence, Drèze and Sen distinguish between two types of successes in the rapid reduction of mortality, which they term "growth-mediated" and "support-led" processes.[62] The former works mainly *through* fast economic growth, exemplified by mortality reductions in places like South Korea and Hong Kong. Their successes depended on the growth process being wide-based and participatory (for example, applying full employment policies) and on the gains from economic growth being utilized to expand relevant social services in the public sector, particularly in health care and education. The experiences of these states stand in stark contrast to the example of countries like Brazil, which have similarly achieved rapid economic growth but lagged behind in health improvements.

In contrast to growth-mediated processes, "support-led" processes operate not through fast economic growth but through governments giving high priority to the provision of social services that reduce mortality and enhance the quality of life. Examples include China, Costa Rica, and the Indian state of Kerala (mentioned above).

A similar choice, between policies emphasizing growth versus those promoting greater equality, applies to developed nations as well. Application of the Rawlsian Difference Principle suggests that a society like the United States has much room to move toward a more equitable (perhaps a more European) distribution of its national income without suffering a loss in productivity or growth. At the same time it would benefit from a gain in the health status of its citizens.

SOME CONCLUDING REMARKS ABOUT BIOETHICS

We noted earlier that bioethics has generally tended to focus on medicine at the point of delivery, attending inadequately to determinants of health "upstream" from the medical system

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

itself. We have tried to remedy that by putting together two elements: empirical findings about the social determinants of health, and the result of a philosophical attempt to construct a theory of justice that might apply to any society. We arrive at the result that social justice, as defined by that theory, is good for our health. In a society that complies with its principles of justice, health inequalities will be minimized and population health status will be improved. A theory of social justice turns out to be a theory about how to distribute health status in a just way, at least if the social science is correct.

The failure of bioethics to look at the social determinants of population health is not primarily a philosophical failing, nor is it simply disciplinary blindness to the social-science or public-health literature. People in bioethics, like the public more generally, concentrate on medical care rather than social determinants, for complex sociological, political, and ideological reasons that we can only mention here. The public, encouraged by scientists and the media, is fascinated by every new biomedical discovery and has come to believe that most of our "success" in improving population health is the result of exotic science. Vast economic interests benefit from this orientation of public and bioethical attention. Economic incentives for people in bioethics come largely from medicine and the scientific and policy institutions that interact with medical delivery. The idea that scientific medicine is responsible for our health blinds us to socioeconomic inequality as a source of inequity in the realization of opportunity for health across the population. Science, we are told, can rescue us all from our shared biological fate, and so we should all unite in supporting a focus on medicine and, if we care about justice, on the equitable access of all to its benefits. Challenging deeper inequalities in society, however, is divisive, not unifying; it threatens those with the greatest power and the most to lose. In the absence of well-organized social movements capable of challenging that inequality, the complaints of public-health advocates pointing the need for more basic change, rather than simply joining existing forces asking for more and better medical care, can seem utopian.

There may be more here than an extension of the scope of Rawlsian moral theory. Earlier, we suggested that challenging

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

Rawls's construct to address questions about the fair distribution of the social determinants of health might show the theory to be generalizable in fruitful ways. This generalizability is analogous to the increase in scope and power of a nonmoral theory when we discover that it can explain phenomena beyond the domain for which its laws were initially developed. What exactly is the analogy?

When an empirical theory turns out to explain new phenomena that were not part of the evidentiary base for its laws, we tend to conclude that the concepts incorporated in it are "projectible" in a desired way. We may think of them as better confirmed as a way of dividing up or describing the world. Rawls begins with certain political concepts thought crucial to our well-being—to meeting our needs as free and equal citizens of a democracy. Using these ideas, the goal was to identify terms of cooperation that all free and equal citizens could agree are fair and reasonable. It then turns out, given the social-science literature, that the aspects of well-being captured by these ideas expand to include the health of the population as well. Whatever controversy might have been thought to surround some of these political components of well-being, they do connect—albeit empirically—to some incontrovertibly objective components of our social well-being, namely, the health of the population. If this were an empirical rather than a normative theory, we would think the evidence of projectibility counted in its favor, constituting support for the theory. Is there additional "support" for Rawls's theory?

It turns out that the support we now give the theory might not be greatly reduced even if the facts were different about the relationship between socioeconomic inequality and health. However, that does not mean we should not add to the support we think the theory has if it turns out to have the projectibility described earlier. A lack of evidence of greater projectibility is not evidence against a theory; it is and should be a neutral finding. If, however, we found that population health was undermined by greater political well-being of the sort the original theory talked about (before its extension), then we would have a puzzle to address: why is one aspect of our well-being working in opposition to other elements of it?

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

This discussion is admittedly too brief to establish a firm conclusion, but we are inclined to think there is some corroborative support for Rawls's theory in the fact that it generalizes to the phenomena of population health in this way. It is the coherence among these different areas of evidence and principle that gives us grounds for thinking the theory has additional justification.[63] This does not mean, of course, that standing objections to the theory, which we have deliberately not addressed, should be ignored.

Another lesson is illustrated by our results. Ethical inquiry is not just one kind of inquiry involving one type of method. This is true for inquiry into practical ethical issues, including bioethics. Sometimes the inquiry requires the importation of tools from political philosophy, from ethical theory, and from the social sciences (as we have done). Sometimes, depending on the problem, we make better progress by examining specific cases carefully and then trying to move to the level of principles and theory. Sometimes we are better off deploying theory or theory-based considerations and refining and adapting them in light of what we learn about particular cases and social science (as we have also done). Since ethical inquiry answers many different kinds of questions, this variability in appropriate method should not surprise us.

Since it is the overall coherence of our system of beliefs that provides us with justification for specific parts of it, we should not become enamored of the tools developed for or appropriate to specific aspects of ethical inquiry and overgeneralize about their importance to all inquiry. In some areas of inquiry in bioethics (or ethics more generally), progress is doomed if we remain insensitive to the local texture of a problem, including the way in which a particular society's beliefs play a role in its policies. But this observation should not lead us to question the relevance of reasons, principles, or theory that purports to bear on issues more universally, despite local texture. We risk impoverishing our inquiry if we insist on limiting ourselves to tools that are best designed for certain specific tasks when our project involves integrating those tasks with others.[64]

No doubt an examination of what policy regarding the distribution of health in a particular population is fair or just will

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

have to take into account some aspects of local beliefs and culture. The same inquiry into policy must also attend to the general implications of social science and political philosophy where they frame the problems raised by local policy.

ACKNOWLEDGMENTS

Norman Daniels, Bruce P. Kennedy, and Ichiro Kawachi are recipients of Robert Wood Johnson Foundation investigator awards in health-policy research.

ENDNOTES

[1]Louis Rene Villerme, *Tableau de l'État Physique et Moral des Ouvriers*, vol. 2 (Paris: Renourard, 1840). Cited in Bruce G. Link et al., "Social Epidemiology and the Fundamental Cause Concept: On the Structuring of Effective Cancer Screens by Socioeconomic Status," *Milbank Quarterly* 76 (1998): 375–402. Throughout, we view disease and disability as departures from (species-typical) normal functioning and view health and normal functioning as equivalent.

[2]Michael Marmot, "Social Causes of Social Inequalities in Health," Harvard Center for Population and Development Studies, Working Paper Series 99.01, January 1999.

[3]To avoid additional complexity, in this essay we concentrate on class or socioeconomic inequalities, though many of our points generalize to race and gender inequalities in health as well.

[4]Goran Dahlgren and Margaret Whitehead, *Policies and Strategies to Promote Social Equity in Health* (Stockholm: Institute of Future Studies, 1991). Michaela Benzeval, Ken Judge, and Margaret Whitehead, eds., *Tackling Inequalities in Health: An Agenda for Action* (London: King's Fund, 1995). Sarah Marchand, Daniel Wikler, and Bruce Landesman, "Class, Health, and Justice," *Milbank Quarterly* 76 (1998): 449–468.

[5]Benzeval, Judge, and Whitehead, eds., *Tackling Inequalities in Health: An Agenda for Action,* 1–9.

[6]John Rawls, *A Theory of Justice* (Cambridge, Mass.: Belknap Press of Harvard University Press, 1971). Norman Daniels, "Health-care Needs and Distributive Justice," *Philosophy and Public Affairs* 10 (1981): 146–179. Daniels, *Just Health Care* (New York: Cambridge University Press, 1985).

[7]Rawls, *A Theory of Justice*, sec. 12–13; Norman Daniels, "Rawls's Egalitarianism," in Samuel Freeman, ed., *Companion to Rawls* (in press).

[8]Marmot, "Social Causes," 6–7; Michael G. Marmot, "Social Differentials in Health Within and Between Populations," *Dædalus* 123 (4) (1994): 197–216.

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

[9]Douglas Black et al., *Inequalities in Health: The Black Report; The Health Divide* (London: Penguin Group, 1988). George Davey-Smith, Martin J. Shipley, and Geoffrey Rose, "Magnitude and Causes of Socioeconomic Differentials in Mortality: Further Evidence from the Whitehall Study," *Journal of Epidemiology and Community Health* 44 (1990): 265–270. Gregory Pappas et al., "The Increasing Disparity in Mortality between Socioeconomic Groups in the United States, 1960 and 1986," *New England Journal of Medicine* 329 (1993): 103–109. Nancy E. Adler et al., "Socioeconomic Status and Health: The Challenge of the Gradient," *American Psychologist* 49 (1994): 15–24.

[10]Davey-Smith, Shipley, and Rose, "Magnitude and Causes of Socioeconomic Differentials in Mortality." Michael G. Marmot et al., "Contribution of Psychosocial Factors to Socioeconomic Differences in Health," *Milbank Quarterly* 76 (1998): 403–408.

[11]Bruce P. Kennedy et al., "Income Distribution, Socioeconomic Status, and Self-rated Health: A U.S. Multi-level Analysis," *British Medical Journal* 317 (1998): 917–921. John W. Lynch et al., "Income Inequality and Mortality in Metropolitan Areas of the United States," *American Journal of Public Health* 88 (1998): 1074–1080.

[12]Richard G. Wilkinson, "Income Distribution and Life Expectancy," *British Medical Journal* 304 (1992): 165–168; "The Epidemiological Transition: From Material Scarcity to Social Disadvantage?" *Dædalus* 123 (4) (1994): 61–77.

[13]Wilkinson, "Income Distribution and Life Expectancy"; Wilkinson, *Unhealthy Societies: The Afflictions of Inequality* (London: Routledge, 1996).

[14]Bruce P. Kennedy, Ichiro Kawachi, and Deborah Prothrow-Stith, "Income Distribution and Mortality: Test of the Robin Hood Index in the United States," *British Medical Journal* 312 (1996): 1004–1008. Published erratum appears in *British Medical Journal* 312 (11 May 1996): 1194. George A. Kaplan et al., "Inequality in Income and Mortality in the United States: Analysis of Mortality and Potential Pathways," *British Medical Journal* 312 (1996): 999–1003.

[15]Lynch et al., "Income Inequality and Mortality in Metropolitan Areas of the United States."

[16]Wilkinson, *Unhealthy Societies: The Afflictions of Inequality*.

[17]Ichiro Kawachi, Bruce P. Kennedy, and Richard G. Wilkinson, eds., *Income Inequality and Health: A Reader* (New York: The New Press, forthcoming). Kaplan et al., "Inequality in Income and Mortality in the United States."

[18]Kennedy et al., "Income Distribution, Socioeconomic Status, and Self-rated Health."

[19]Kimberly Lochner, "State Income Inequality and Individual Mortality Risk: A Prospective Multilevel Study," Ph.D. dissertation, Harvard University, 1999.

[20]Kaplan et al., "Inequality in Income and Mortality in the United States." Ichiro Kawachi and Bruce P. Kennedy, "Health and Social Cohesion: Why Care about Income Inequality?" *British Medical Journal* 314 (1997): 1037–1040.

[21]Ichiro Kawachi et al., "Women's Status and the Health of Women: A View from the States," *Social Science and Medicine* 48 (1999): 21–32. Richard G.

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

Wilkinson, Bruce P. Kennedy, and Ichiro Kawachi, "Women's Status and Men's Health in a Culture of Inequality," *International Journal of Health Services Research* (under review).

[22]Kawachi and Kennedy, "Health and Social Cohesion." Ichiro Kawachi et al., "Social Capital, Income Inequality and Mortality," *American Journal of Public Health* 87 (1997): 1491–1498.

[23]Ichiro Kawachi and Bruce P. Kennedy, "Income Inequality and Health: Pathways and Mechanisms," *Health Services Research* 34 (1999): 215–227. The level of interpersonal trust at the state level was gauged by responses to the General Social Surveys conducted by the National Opinion Research Center, which asked citizens questions such as "Generally speaking, would you say that most people can be trusted or that you can't be too careful in dealing with people?" This approach to measure social trust is the same as the approach adopted by Robert D. Putnam in his work on social capital: "Bowling Alone: America's Declining Social Capital," *Journal of Democracy* 6 (1995): 65–78.

[24]Margaret Whitehead, "The Concepts and Principles of Equity and Health," *International Journal of Health Services* 22 (1992): 429–445. Dahlgren and Whitehead, *Policies and Strategies to Promote Social Equity in Health*. Paula Braveman, *Monitoring Equity in Health: A Policy-oriented Approach in Low- and Middle-income Countries* (Geneva: World Health Organization, 1999).

[25]Daniel Wikler, "Persuasion and Coercion for Health: Issues in Government Efforts to Change Life Style," *Milbank Quarterly* 56 (1978): 303–338. John E. Roemer, *Equality of Opportunity* (Cambridge, Mass.: Harvard University Press, 1998).

[26]Joshua Cohen, "Democratic Equality," *Ethics* 99 (1989): 727–754.

[27]Daniels, *Just Health Care*. John Rawls, *Political Liberalism* (New York: Columbia University Press, 1993).

[28]A careful discussion of Rawls's argument for the Difference Principle and the extensive critical literature it has generated is beyond the scope of this essay. It is important, however, to distinguish Rawls's own social contract argument from the many informal and intuitive reformulations of it. See Brian Barry, *Theories of Justice* (London: Harvester Wheatsheaf, 1989), 213–234; Gerald A. Cohen, "Incentives, Inequality, and Community," in *The Tanner Lectures on Human Values*, vol. 13, ed. G. B. Petersen (Salt Lake City: University of Utah Press, 1992), 262–329. Gerald A. Cohen, "The Pareto Argument for Inequality," *Social Philosophy and Policy* 12 (Winter 1995): 160–185. Cohen, "Democratic Equality," 727–751. Daniels, "Rawls's Egalitarianism."

[29]Rawls, *A Theory of Justice*.

[30]Amy Gutmann and Dennis Thompson, *Democratic Disagreement* (Cambridge, Mass: Harvard University Press, 1995).

[31]Rawls, *A Theory of Justice*, 62; *Political Liberalism*, chap. 5.

[32]This example is taken from Joshua Cohen, "The Pareto Argument," unpublished manuscript.

[33]Daniels, "Rawls's Egalitarianism."

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

[34]Daniels, *Just Health Care*. Norman Daniels, Donald W. Light, and Ronald L. Caplan, *Benchmarks of Fairness for Health Care Reform* (New York: Oxford University Press, 1996), 41–44.

[35]Norman Daniels, "Mental Disabilities, Equal Opportunity, and the ADA," in Richard J. Bonnie and John Monahan, eds., *Mental Disorder, Work Disability, and the Law* (Chicago: University of Chicago Press, 1996), 282–297.

[36]Daniels, *Just Health Care,* chaps. 7–8.

[37]Cohen ("Incentives, Inequality, and Community") has argued that a *strict* interpretation of the Difference Principle would allow few incentive-based inequalities; for a more permissive view, see Daniels, "Rawls's Egalitarianism."

[38]Daniels, *Just Health Care,* chap. 7.

[39]Kenneth Arrow, "Uncertainty and the Welfare Economics of Medical Care," *American Economic Review* 53 (1963): 941–973.

[40]Amartya K. Sen, "Equality of What?" in Sterling M. McMurrin, ed., *Tanner Lectures on Human Values*, vol. 1 (Cambridge: Cambridge University Press, 1980). Amartya K. Sen, *Inequality Reexamined* (Cambridge, Mass.: Harvard University Press, 1992).

[41]Norman Daniels, "Equality of What: Welfare, Resources, or Capabilities?" *Philosophy and Phenomenological Research* 50 (1990): 273–296.

[42]Rawls, *A Theory of Justice,* 243–251.

[43]Rawls does suggest, since fair equality of opportunity is given priority over the Difference Principle, that within the index, we can assume opportunity has a heavier weighting. Rawls, *A Theory of Justice*, 93.

[44]James Fishkin, *Justice, Equal Opportunity, and the Family* (New Haven, Conn.: Yale University Press, 1983).

[45]Norman Daniels and James E. Sabin, "Limits to Health Care: Fair Procedures, Democratic Deliberation, and the Legitimacy Problem for Insurers," *Philosophy and Public Affairs* 26 (1997): 303–350.

[46]Richard J. Arneson, "Equality and Equal Opportunity for Welfare," *Philosophical Studies* 54 (1988): 79–95. Gerald A. Cohen, "On the Currency of Egalitarian Justice," *Ethics* 99 (1989): 906–944. Roemer, *Equality of Opportunity*, chap. 12–13.

[47]For Sen's account, see Sen, *Inequality Reexamined*, chaps. 1–5, 9. For the account offered by Daniels and others, see Daniels, "Equality of What: Welfare, Resources, or Capabilities?"; A. Buchanan et al., *From Chance to Choice: Genetics and the Just Society* (New York: Cambridge University Press, in press).

[48]Elizabeth Anderson, "What is the Point of Equality?" *Ethics* 109 (2) (January 1999): 287–337; cf. Daniels, "Rawls's Egalitarianism."

[49]John N. Lavis and Gregory L. Stoddart, "Can We Have Too Much Health Care?" *Dædalus* 123 (4) (1994): 43–60.

[50]Ibid.

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms

[51]Lawrence J. Schweinhart, Helen V. Barnes, and David P. Weikart, *Significant Benefits: The High/Scope Project Perry Preschool Study through Age 27* (Ypsilnati, Mich.: The High/Scope Press, 1993).

[52]Donald Acheson et al., *Report of the Independent Inquiry into Inequalities in Health* (London: Stationery Office, 1998).

[53]Nancy E. Moss and Karen Carver, "The Effect of WIC and Medicaid on Infant Mortality in the United States," *American Journal of Public Health* 88 (1998): 1354–1361.

[54]Michael G. Marmot et al., "Contribution of Job Control and Other Risk Factors to Social Variations in Coronary Heart Disease Incidence," *Lancet* 350 (1997): 235–239.

[55]Robert A. Karasek and Töres Theorell, *Health Work* (New York: Basic Books, 1990), 83–157.

[56]Acheson et al., *Report of the Independent Inquiry into Inequalities in Health,* 48–50.

[57]Kawachi et al., *Income Inequality and Health.*

[58]Kawachi and Kennedy, "Health and Social Cohesion"; "Income Inequality and Health."

[59]Kim Quaile Hill, Jan E. Leighley, and Angela Hinton-Andersson, "Lower-class Mobilization and Policy Linkage in the U.S. States," *American Journal of Political Science* 39 (1995): 75–86.

[60]Kawachi et al., *Income Inequality and Health,* xxviii-xxx.

[61]Amartya K. Sen, "Mortality as an Indicator of Economic Success and Failure," *The Economic Journal* 108 (1998): 1–25.

[62]Jean Drèze and Amartya Sen, *Hunger and Public Action* (Oxford: Clarendon Press, 1989), 183–187.

[63]Norman Daniels, "Wide Reflective Equilibrium and Theory Acceptance in Ethics," *Journal of Philosophy* 76 (1979): 256–282.

[64]Daniels, *Justice and Justification,* 333–352.

This content downloaded from
23.93.189.118 on Thu, 19 Mar 2026 15:13:29 UTC
All use subject to https://about.jstor.org/terms